Family Child Care Parent Handbook Program Name:

Helena Street Day Care Center

Program Address: 171 Helena Street Leominster, MA 01453

Program Phone Number: 9784270302

Program Email:Appiahemma99@gmail.com

Dear Parent,

As a licensed Family Child Care Educator, I would like to congratulate you on choosing licensed Family Child Care. You have made an important childcare decision for you and your family. The Department of Early Education and Care (EEC) and I invite you to join in a partnership with us to ensure a high-quality childcare environment. This parent handbook and enrollment packet outlines many of the policies and procedures that relate to the care of your child, as well as the information I am required to give to you when you enroll your child in **HELENA STREET CHILD CARE**. This handbook will also acquaint you with some of the key EEC standards designed to ensure a safe, healthy and educational childcare experience.

I encourage you to maintain an open dialogue with me, as communication between parents and Educators is the foundation for a solid working relationship, and a good childcare experience. Before filling out your childcare enrollment form, please read through the information contained in this parent handbook.

A Word from EEC

EEC is the agency that oversees the early education and care and after school services for families in Massachusetts. As the agency that licenses childcare, EEC has quality standards for all licensed programs to ensure high educational value, as well as health and safety. Having a license means that I have demonstrated that I meet the standards outlined in the EEC regulations.

To obtain your own copy of EEC Family Child Care Regulations, you may download them from the EEC web site at:
http://www.mass.gov/Eeoe/docs/EEC/regs_policies/20090122_606_cmr.pdf

For information about my regulatory compliance history, you may contact our local EEC regional office, whose contact information is as follows:

324-R Clark Street

Worcester, MA 01660

508 461 1440

Enrollment/Capacity

My current licensed capacity is _____, which is posted on my license. At any one time, I can only care for the number of children that I've been licensed for, which may include my own children, depending on their ages. In addition, EEC regulations state that I cannot care for more than three (3) children under the age of two (2) without an assistant, with one of those children being at least 15 months old and walking unassisted. If you have concerns or questions about the number of children in my care, please feel free to discuss them with me.

Use of Assistants

I may have an assistant to help care for the children at the , provided they are approved by EEC. If and when I use an assistant, I will let you know ahead of time, and you will have an opportunity to meet the assistant that will be working in the program. I may also use volunteers from time to time, and although they will not be directly responsible for the care of children in the program, they will be on the premises and assisting me.

[ ] I am not currently utilizing an assistant.

[ ] The assistant(s) / volunteer(s) I am currently using in my program are:

Name Certificate (for assistants)

_____

_____

Program Hours / Closures (See Attachment A)

Attached to this handbook is a parent / educator agreement that outlines my policies regarding hours of care, late fees and termination. We will review this together and note any additional information that is specific to you and the care of your child(ren).

Sick Policy

I can care for mildly ill children in my program, however there will be times when you will need to keep your children out of the program due to illness. If your child has a fever, diarrhea or vomiting, you should keep them out of care until those symptoms have been resolved for 24 hours. This policy is designed to maintain a healthy environment for all children and to ensure the sustainability of this childcare program. My additional policies regarding caring for ill children are as follows:

In accordance with **Massachusetts EEC Regulation 606 CMR 7.11**, the following terms apply:

1. The 100°F Fever Rule

- A temperature of **100.0°F** or higher is considered a fever.
- Children will be sent home if they reach this threshold.
- **The 24-Hour Reset:** Children must be fever-free and symptom-free for **24 full hours** without the use of fever-reducing medications (e.g., Tylenol, Motrin) before returning to care.

2. Exclusion Criteria (When to Stay Home)

The following symptoms require the child to be excluded from care:

Symptom	**Stay Home / Will Be Sent Home If...**
Fever	Temperature is **100.0°F** or higher.
Vomiting	2 or more episodes within the last 24 hours.
Diarrhea	Watery stools or stools that cannot be contained in a diaper.

Symptom	Stay Home / Will Be Sent Home If...
Respiratory	Severe cough, wheezing, or difficulty breathing.
Eyes	Redness, itching, or yellow/crusty discharge (Pink Eye).
Rash	Any unexplained rash, until cleared by a doctor's note.
Behavior	Child is too lethargic/ill to participate in regular activities.
Communicable	Diagnosis of Strep, HFM, Flu, etc. (Requires 24hrs of antibiotics).

3. Tuition & Payment Policy

To secure your child's placement in this program, tuition is based on a **reserved space**, not on actual attendance.

- **Illness Absences:** Full tuition is due even if your child is absent due to illness.
- **Why this is required:** Operating costs (insurance, mortgage/rent, licensing, and supplies) remain the same regardless of attendance. Your payment ensures that your child's spot is held and not offered to a family on the waiting list.
- **Provider Illness:** If the provider closes the program due to personal illness, tuition will be credited/waived for those days.

4. Medication Administration (MA Regulation)

- I will not administer the **first dose** of any new medication.
- All medications must be in the original pharmacy container with the child's name.
- A signed **Medication Consent Form** must be on file before any dose is given.

5. Parent Notification Requirement

Parents must notify the provider within **24 hours** if a child is diagnosed with a communicable disease. The provider will then notify all enrolled families of the potential exposure (maintaining the sick child's anonymity).

Plan for Meeting Potential Emergencies

EEC regulations require that I have a plan for meeting potential emergencies that may occur either during childcare hours or at any time if they may affect the operation of the program.

In the event of an evacuation emergency, I will contact the local authorities to determine whether or not to evacuate the program, or to remain sheltered at the program.

The escape routes from each floor of the licensed childcare space are as follows:

1. Designated Escape Routes by Floor

This program maintains at least two separate, EEC-approved exits from every floor level used for childcare to ensure safety if one route is blocked.

- **Main Floor Routes:**
    - **Primary Exit:** Evacuation through the **Main Entrance Door** leading to the front yard/street.
    - **Secondary Exit:** Evacuation through the **All-Season Patio** sliding doors/exit leading to the backyard.
    - **Alternative Exit:** Evacuation through the **Garage** to the drive.
        - *Safety Compliance:* The garage door is maintained to ensure it can be opened manually without electricity. The garage is kept unlocked and free of clutter or hazardous chemicals during business hours.
- **Basement Level Routes (Walkout):**
    - **Primary Exit:** Direct evacuation through the **Walkout Basement Door** to the outside ground level backyard.

2. Emergency Meeting Point

Upon exiting the home, all children and staff will immediately proceed to the designated safe meeting spot: **Basketball pool at the end of the driveway**. A head-to-face count will be performed immediately using the daily attendance log.

3. Evacuation Procedures & Mobility

- **The "Go-Bag":** The provider will carry the emergency bag containing the first aid kit, daily attendance sheet, emergency contact forms, and any life-saving medications (EpiPens, Inhalers).
- **Non-Mobile Children:** Infants or children with limited mobility will be evacuated using e.g., an evacuation crib or multi-child stroller kept near the primary exit.
- **Clearance:** All exit paths, including the patio area and garage floor, are kept clear of snow, ice, and debris at all times.

4. Monthly Drills (EEC Requirement)

Per **606 CMR 7.11(7)(h)**, we perform and document monthly evacuation drills.

- We rotate our practice sessions to include **all** approved exits: the Main Door, the All-Season Patio, the Garage, and the Walkout Basement.
- Drills are conducted under varying conditions (e.g., during nap time or mealtimes) to ensure readiness.

In the event that a child goes missing from the program, I will do the following:

Missing Child Protocol & Prevention

1. Prevention & Active Supervision

- **Face-to-Name Tracking:** Staff will match each child's face to their name on the attendance sheet at every transition and at 15-minutes intervals.
- **Transition Log:** A log is updated every time children move between floors or areas e.g. Moving from the inside of the main house to either front yard or backyard.
- **Site Security:** All exits (Main, Patio, Garage, Basement) remain locked from the inside. Visitors must be verified by photo ID and escorted by staff at all times.
- **Authorized Pickup:** Children are only released to individuals on the authorized list. Sign-out is performed exclusively by the **Program Director** or **FCC Assistant**.

2. Emergency Response: Missing Child

In the event a child is determined to be missing, the following steps will be taken **immediately**:

1. **Call 911:** State: *"I am a licensed childcare provider at **171 Helena Street Leominster, MA 01453**. I have a missing child."* Provide name, age, description, and the last place they were seen.
2. **Contain Other Children:** Move all other children **indoors** immediately. They must be contained in a single classroom/room under the active supervision of a caregiver or the **Emergency Back-up Person**.
3. **Search the Premises:**
    - Search the immediate area where the child was last seen.
    - Thoroughly sweep the interior, including closets, under furniture, and bathrooms.

- Check all exterior "transition" zones: the **All-Season Patio**, the **Garage**, and the perimeter of the **Walkout Basement**.
- If off-site, check all **vehicles** (under seats and in trunks).

4. **Notify Parents:** Call the missing child's parents/guardians immediately after the 911 call.
5. **Emergency Back-Up:** If the Provider is alone, the designated Emergency Person will be called to take over the care of the remaining children until all parents can arrive for pickup.
6. **Mandatory EEC Report:** The Provider will notify the **EEC Licensor within 24 hours** of the incident and formally notify the parent/guardian in writing within 24 hours.

Should the program need to be evacuated in the case of a fire, natural disaster, loss of power, heat or hot water, or any other emergency situation, we will meet at an alternate location. The designated meeting place(s) outside of the program for emergencies are:

Leominster High School -122 Granite St, Leominster, MA 01453.

If the program needs to be evacuated, I will notify all parents, as well as the appropriate authorities (fire, police, etc.) and EEC. My method of doing that is as follows be cell phone call from 978-427-0302. Alternative phone number: 978-73-8219.

I will ensure that no child has been left at the program after an evacuation by:

Post-Evacuation Verification & Clearance

To ensure that no child has been left at the program site after an evacuation, the following protocol will be strictly followed:

1. **The "Face-to-Name" Headcount:**
   - Immediately upon reaching the designated meeting spot **Leominster High School, 122 Granite St** Leominster MA, the Provider/Assistant will conduct a formal headcount.
   - This is not a "head count" by number, but a **Face-to-Name match** using the physical attendance sheet to ensure every specific child is present.
2. **Comprehensive House Sweep:**
   - Before exiting the building, the Provider (or a designated Assistant) will conduct a rapid but thorough sweep of the licensed childcare space.

- **High-Risk Areas:** Special attention will be paid to "hiding spots" where children may go if they are frightened by alarms, including:
    - Inside all **bathrooms** (behind toilets and shower curtains).
    - Inside all **closets** and under all **beds/cribs**.
    - Behind furniture in the **all-season patio** and **walkout basement**.
    - The interior of the **garage**.

3. **Final Portal Check:**
    - The Provider will verify that all interior doors are closed (to slow the spread of fire) and that the final exit door is latched behind them.
4. **Emergency "Go-Bag" Verification:**
    - The Provider will confirm that the emergency bag, containing the medical binder, daily attendance sheet, and contact information, has been successfully removed from the building.

Summary of Your Meeting Places
• **Primary Meeting Place (Immediate):** Basketball pole at the end of the driveway.
• **Secondary Meeting Place (Neighborhood): Leominster High School**, 122 Granite St, Leominster, MA 01453.

Children's Records

EEC regulations require **HELENA STREET CHILD CARE** to maintain an individual written record for every child I have in care. These records include the information that **parents complete at enrollment**, as well as **progress reports**, **incident reports** and other documentation regarding your child's care. Records are updated at least annually but may be updated as frequently as is needed.

As a parent, you have access to the record that I maintain for your child, and you have the right to add information or to request that information in your child's record be changed or deleted. **You also have a right to receive a copy of your child's record; however, I may charge a reasonable fee for that copy**.

EEC regulations require that I make children's records available to EEC at any time that EEC may request, such as during a licensing/monitoring visit, a complaint investigation, or a financial review of my program. Failure on my part to provide these records to EEC could result in EEC citing me for regulatory non-compliance or taking legal action against my

license. When EEC staff members review children's records to ensure that I am in compliance with EEC regulations, at times they may copy and keep the information found in these records in order to review my compliance with all EEC regulations and policies applicable to my program. This information will be kept in my EEC Licensing file or in EEC's financial monitoring file if the information involves issues related to subsidized care. EEC is required by law to keep confidential any personally identifiable information found in children's records collected and maintained by EEC staff members. EEC has a Privacy Policy which discusses how EEC keeps such information confidential. That policy can be found by going to the EEC website at http://www.eec.state.ma.us/docs1/20101124_eec_privacy_policy.pdf.

Please let me know about any questions you have regarding your child's record.

Maintaining a Safe Environment

EEC has a number of licensing standards related to safety in a Family Child Care Home. Most of these standards outline common safety precautions such as making dangerous materials inaccessible to children, covering outlets, having a first aid kit, practicing evacuation drills, gating stairs, windows, or heating elements, posting emergency numbers, and maintaining a clean, hazard-free indoor space. Also, the outdoor space must be safe and hazard free and there should be no access to a busy street, water, construction materials, rusty or broken play materials, debris, glass, or peeling paint.

Lead Poisoning Prevention

All Family Child Care Educators are required by EEC to provide parents with information regarding the risks of Lead Poisoning. The following are some facts that all parents should know about lead and lead poisoning:

• Lead poisoning is caused by swallowing or breathing lead. Lead is poison when it gets into the body.

• Lead can stay in the body for a long time. Young children absorb lead more easily than adults. The harm done by lead may never go away.

Lead in the body can:

1. Hurt the brain,
2. kidneys, and nervous system
3. Slow down growth and development
4. Make it hard to learn
5. Damage hearing and speech
6. Cause behavior problems

• Most of the lead poisoning in Massachusetts comes from lead paint dust in older homes. Many homes built before 1978 have lead paint on the inside and outside of the building.

• When old paint peels and cracks, it creates lead paint chips and lead dust. Lead dust also comes from opening and closing old windows.

• Lead dust lands on the floor. Lead gets into children's bodies when they put their hands and toys in their mouths. Children can also breathe in lead dust. Children between the ages of 9 months and 6 years are most at risk.

• Important: Home repairs and renovations also create lead dust.

• Most children who have lead poisoning do not look or act sick. A lead test is the only way to know if your child has lead poisoning. Ask your doctor to test your child for lead. Some children may have:

1. Upset stomach
2. Trouble eating or sleeping
3. Headache
4. Trouble paying attention

As mentioned earlier, if your child is over nine (9) months of age, you will need to provide documentation to **HELENA STREET CHILD CARE** that your child has been screened for lead poisoning. Most children will be screened annually until either age three (3) or four (4), depending on where the child lives.

I am required to disclose to you if I am aware of any known sources of lead in my home. Information regarding known sources of lead in my home is as follows:

Lead Paint Disclosure: I hereby certify that I have no knowledge of any lead-based paint or lead-based paint hazards currently present within the childcare environment. I have not received any notification or reports from the Department of Public Health (DPH) or local boards of health indicating the presence of lead hazards at this location.

Recent Renovations and Maintenance: To ensure a safe and healthy environment for the children in my care:

The facility underwent extensive interior and exterior renovations and repainting in 2024.

All painted surfaces are currently in good repair, free of any chipping, flaking, or peeling paint, as required by EEC safety standards.

For more information on lead poisoning, you can visit http://www.mass.gov/dph/clppp or call the Childhood Lead Poisoning Prevention Program at (800) 532-9571.

Supervision

Supervision is critical to keeping children safe. I and any assistants in my program will appropriately supervise children in order to ensure their health and safety at all times. I will use good judgment and consider several factors in determining the appropriate level of supervision for children including age, developmental needs, behavioral characteristics, the nature of activities and the space we are using, as well as the number of caregivers present at any given time. If you have any questions about how I supervise the children in my program, feel free to ask me.

Safe Sleep

Supervision of children is equally important during the times that a child is sleeping at the program, particularly when that child is an infant. EEC has very specific regulations around safe sleep practices. All infants are placed on their backs to sleep, unless a child's physician orders otherwise **(such an order must be given to me in writing).** I check on **children every 15 minutes during naptime**. If your child is less than six months old, I will directly supervise them during **naptime for the first six weeks they are in care**. For more information regarding Safe Sleep, please feel free to review the 'Family Child Care Policies' section of www.eec.state.ma.us.

Curriculum and Progress Reports

All Family Child Care Educators must carry out a routine that is flexible and responds to the needs and interests of children in care. The routine must include things such as; meeting the physical needs of children in care, sixty minutes of physical activity every day, child-initiated and Educator-initiated activities and daily outdoor play, weather permitting. Additionally, the Educator must develop a curriculum that engages children in developmentally appropriate activities by planning specific learning experiences. The curriculum must include things such as; **learning self-help skills that foster independence**, **opportunities to gain problem solving** and **decision-making competencies and leadership skills and opportunities** to learn about **proper nutrition, good health and personal safety**. I am also responsible for providing an environment that promotes cultural, social and individual diversity.

In addition, progress reports must be completed periodically for all children in care. For infants and children with identified special needs, I will be completing progress reports every three months. For toddlers and preschoolers, those reports are completed every six months, and school-age children will have a yearly progress report completed for them.

I will be sharing your child's progress reports with you, as well as offering an opportunity to meet and discuss your child's progress. Feel free to ask me about curriculum and progress reports and how they are implemented in my program.

Child Guidance

When it comes to interactions and the guiding of children's behavior, the goal of all Educators is to maximize the growth and development of children, as well as keep them safe. My Child Guidance Policy is as follows:

1. Our Philosophy

The goal of all Educators in this program is to maximize the growth and development of children while ensuring their physical and emotional safety. We believe that child guidance is a continuous process of teaching, not punishment. We use positive techniques that foster self-esteem, self-control, and social competence.

2. Positive Guidance Techniques

We utilize the following methods to encourage appropriate behavior:

- Positive Reinforcement: We recognize and reinforce children's appropriate behaviors and accomplishments through verbal encouragement and descriptive praise.
- Redirection: When a child engages in challenging behavior, we gently redirect their energy toward a more positive activity or alternative choice.
- Environment & Routine: We arrange the physical space to prevent boredom, and conflict, and we maintain a predictable daily routine so children feel secure.
- Modeling: Educators model courtesy, respect, and non-violent conflict resolution at all times.
- Setting Clear Limits: We set clear, consistent, and age-appropriate expectations. We explain the "why" behind rules, so children understand the reason for safety.
- Choices: We offer meaningful choices (e.g., "Would you like to clean up the blocks or the books first?") to give children a sense of autonomy and cooperation.

3. Use of "Time-Out"

In accordance with 606 CMR 7.05(8)(f), if a "time-out" or brief separation from the group is necessary:

- It will not exceed one minute for each year of the child's age.

- The child will always remain within the Educator’s view and hearing.
- The purpose is to allow the child to regain self-control, after which the Educator will help the child transition back to the group.

4. Prohibited Practices (State Mandated and program prohibited)

The following practices are strictly prohibited in this program:

- Corporal Punishment: No spanking, hitting, shaking, or any other physical punishment.
- Cruel or Severe Punishment: No humiliation, verbal abuse, neglect, or derogatory remarks.
- Deprivation: Children will never be deprived of meals, snacks, or outdoor time as punishment.
- Force Feeding: Children will never be forced to eat against their will or have food used as a consequence.
- Toileting: No child will be punished or shamed for soiling, wetting, or not using the toilet. No child will be forced to remain in soiled clothing or on the toilet.
- Physical Restraint: We do not use any form of physical restraint to manage behavior.

Child Guidance: Maximizing Growth & Development

When it comes to interactions and the guiding of children’s behavior, the goal of all Educators is to maximize the growth and development of children, as well as keep them safe. Our approach focuses on the following developmental pillars:

1. Emotional Intelligence & Literacy

To help children grow emotionally, we don’t just stop "bad" behavior; we help them identify the feeling behind it.

- Naming the Feeling: We help children label their emotions (e.g., "I see you are feeling frustrated because the tower fell").
- The "Calm Down" Space: We provide a dedicated, cozy space where children can go voluntarily to regulate their emotions using sensory tools or books. This is not a punishment, but a skill for self-regulation.

2. Social Problem-Solving

We maximize development by involving children in the resolution process rather than just handing out a solution.

- Mediated Conflict: When two children want the same toy, the Educator acts as a facilitator, asking questions like, "What is your plan for when he is done with that?"
- Natural Consequences: Whenever safe, we allow natural consequences to teach life lessons (e.g., "If we don't put the caps on the markers, they will dry out and we won't be able to color later").

3. Executive Function & Independence

Growth is maximized when children feel in control of their environment.

- Predictable Routines: Our daily schedule is posted with pictures so children can anticipate transitions, reducing anxiety-based behaviors.
- Environment Design: Toys and materials are stored on low, accessible shelves in the All-Season Patio and Basement play areas to encourage independent choice and responsibility for clean-up.

4. Positive Communication & Encouragement

We focus on "Growth Mindset" language.

- Process Praise: Instead of saying "Good job," we say, "I see how hard you worked to balance those blocks!" This encourages persistence and effort.
- The "Do" Rule: We tell children what to *do* rather than what *not* to do (e.g., "Please use walking feet" instead of "Don't run").

Educator Interactions Standard

To maintain a safe and nurturing environment, all Educators and Assistants agree to:

- Interact at Eye Level: We crouch or sit to speak with children at their level to minimize intimidation and build trust.
- Active Listening: We give children our full attention when they are speaking, validating their thoughts and ideas.
- Consistent Expectations: We maintain the same boundaries across all areas of the home, so children feel secure in knowing the "rules of the road."

Medication Administration

EEC has regulations requiring Educators to have a policy regarding the administration of medication to children in care. As a licensed Family Child Care Educator, I am also required to take medication administration training. The following guidelines are common to all programs that are licensed by EEC:

Prescription Medication

First dose of prescription medication must be administered by parent/ guardian to child.

• Prescription medication must be brought to the program in its original container and include **the child's name, the name of the medication, the dosage, the number of times per day and the number of days the medication is to be administered.** This prescription label will be accepted as the written authorization of the physician.

• The program will not administer any medication contrary to the directions on the label unless so authorized by written order of the child's physician.

• The parent must fill out the Authorization for Medication Form before the medication can be administered.

Non-prescription Medication

• The program needs written parental authorization to administer oral non-prescription medication. The parent must fill out the Authorization for Medication form, which allows the Educator to administer the non-prescription medication. The statement must be renewed on a weekly basis.

• In the case of unanticipated non-prescription medication that is used to treat mild symptoms (e.g., acetaminophen, ibuprofen), the program must still have written parental authorization, however it must be reviewed annually.

• The Educator will make every attempt to contact the parent prior to the child receiving the non-prescription medication unless the child needs medication urgently or when contacting the parent will delay appropriate care unreasonably.

Topical Ointments and Sprays

• Topical ointments and sprays such as petroleum jelly, sunscreen, diaper rash ointment and insect repellant will be administered to the child with written parental permission. The signed statement from the parent will be valid for one year and include a list of topical non-prescription medication.

• When topical ointments and sprays are applied to wounds, rashes, or broken skin, the Educator will follow the written procedure for non-prescription medication which includes the written order of the physician, which is valid for a year, and the Authorization for Medication form signed by the parent.

All Medications

• The first dose must be administered by the parent at home in case of an allergic reaction.

• All medications must be given to the Educator directly by the parent.

• All medications will be stored out of the reach of children. All medications that are considered controlled substances must be locked and kept out of reach of children.

• The Educator will be responsible for the administration of medication. In his/her absence, the designated person will administer medication.

• The program will maintain a written record of the administration of any medication (excluding topical ointments and sprays applied to unbroken skin) which will include the child's name, the time and date of each administration, the dose, and the name of the person administering the medication. This completed record will become part of the child's file.

• All unused medication will be returned to the parent if possible or disposed of in accordance with Department of Public Health guidelines.

Oral Health

Proper oral health begins at home, and I will be reinforcing good oral health practices with your child each day. If your child is in care for more than 4 hours per day, or he/she will be receiving at least one meal while in care, I am required to assist your child with toothbrushing at the program if requested by the parent.

I would appreciate it if you would provide me with a toothbrush and toothpaste for your child, which I will store in a safe and sanitary manner at the program.

Parent/guardians will be providing tooth brushing materials for their child the program.

Parent Notifications

I am required by EEC regulations to notify you of certain information about my family childcare home. These notifications include, but are not limited to:

• an injury to your child;

• allegations of abuse or neglect regarding your child;

• if another educator will be caring for your child;

• the administering of first aid to your child;

• whenever a communicable disease has been identified in the program;

• children being taken off the child care premises;

• the existence of firearms in my home;

• if there are any changes in my household composition,

• prior to any pets being introduced into the program;

• whenever special problems or significant developments arise.

Mandated Reporting

As a licensed Educator in Massachusetts, I must operate my program in a way that protects children from abuse and neglect. As such, I am a mandated reporter (under M.G.L. c.119 s51A) and must make a report to the Department of Children and Families (DCF) whenever I have reasonable cause to believe a child in the program is suffering from a serious physical or emotional injury resulting from abuse inflicted upon the child, or from neglect, no matter where the abuse or neglect may have occurred or by whom it was inflicted.

What I Need from You

The first day your child attends childcare, I need a copy of the attached **Family Child Care Enrollment Packet**. Without these completed documents, which must be updated annually, I cannot care for your child. The reason for this is so I have all the important information and phone numbers I will need to provide the best possible care for your child.

Medical Information

Medical information about your child must be given to me within one (1) month from the day your child begins care. There are three (3) pieces of medical information I will need:

1. A statement from a physician or health care professional that says that your child received a physical exam within the past year;

2. Evidence that your child has been immunized as recommended by the Department of Public Health;

3. If your child is nine (9) months of age or older, a statement from a physician or health care professional which says that your child has been screened for lead poisoning.

Please note: Your child's immunization record must be updated and given to me in accordance with the Department of Public Health's immunization schedule. Also, your child's lead screening report must be updated as required by Department of Public Health Regulations. This report must also be given to me. If your child is school age, I can accept a written statement that the required information is on file with the child's school.

Communication and Staying Involved

It is important to keep an open dialogue with me as your child's Educator, and to maintain an active role in your child's care. Feel free to visit, not just at pick up and drop off time, but at a variety of times during your child's day—it's your right as a parent. Please also make sure to follow-up with me if you have any questions about the program or your child's care.

I look forward to working with your family and providing a great experience for your child(ren)!

ATTACHMENT A---PARENT / EDUCATOR AGREEMENT

Program Name: HELENA STREET CHILD CARE

Name of Child(ren):_____

Program Address: 171 HELENA STREET LEOMINSTER, MA 01453

Program Hours / Closures

My normal hours of operation:

Helena Street Childcare will be open Monday through Friday from 7:00AM to 7:00PM. Arrangement must be made with provider for WEEKEND care. To ensure a consistent routine for the children and appropriate staffing levels, the following arrival and departure times are established:

- Standard Drop-off: 7AM
- Standard Pick-up: 3:00PM to 3:30PM.

The hours your child will be taking care of are as follows: 7AM to 3:00PM

Extended Care & Late Fees

We understand that families may occasionally require care beyond the standard 3:30PM pick-up time. Parents/guardians must notify program if extended care will be needed before pick-up time. To accommodate these needs while covering additional operational costs, the following fees apply:

1. Occasional Extended Care (Daily Fee) Parents who pick up their child between 4:10PM and 7PM on an unscheduled or occasional basis will be charged an additional fee **of $40.00 per day**. This fee is due at the time of pick-up or will be added to the following week's invoice.
2. Scheduled Extended Care (Weekly Flat Rate) Families who require a consistent pick-up time between 4:00PM and 6:45PM may opt into our Extended Care program for a flat rate of **$80.00 per week** in addition to standard tuition. Note: **This option must be arranged in advance and noted in the children's enrollment contract.**
3.

If for any reason the program is closed, I will notify you by: **cell phone**

Additional schedule information (holidays, training days, vacation, etc.) is as follows:

Will be provided to you at least 48 hour prior to any holiday, vacation, or training so appropriate arrangement(s) could be made.

Care Type	**Suggested Weekly Rate**	**Daily Rate (Drop-in/Part-time)**
Infant (0-15 months)	**$325 – $375**	$75 – $85
Toddler (15 mo - 2.9 yrs)	**$300 – $340**	$65 – $75
Preschool (2.9 - 5 yrs)	**$275 – $315**	$60 – $70

3. Recommended Late Fee & Termination Policies

- **Termination Policy:** * **Provider Termination:** Reserve the right to terminate immediately for non-payment or safety issues.
    - **Parent Termination:** Require **2 to 4 weeks' written notice**. Many providers require the final 2 weeks to be paid in full regardless of attendance.

4. Important Compliance Tip

By signing this, you are documenting that we are in agreement about the hours your child will receive care, and the rates you will be paying for that care. You are also stating that you understand my policies regarding late fees, termination, and any other issues documented above.

Parent Signature Date